

Required Purpose (98.0%):

Medical Billing & Practice Management Software Boost efficiency and collections with our practice management software and services.

Data Controller (93.3%), Non-Personal Data (80.5%), Organisational Measure (65.6%), Required Purpose (95.6%), Retention (73.8%), Third Party (77.2%):

Disclaimers, Terms and Conditions

athenahealth Patient Login athenaOne Login Solutions Solutions Who We Serve Who We Serve Resources Resources Company Company Request a Demo Request a Demo Solutions Our PlatformathenaOne An AI-powered, all-in-one solution that improves clinical effectiveness, patient experience, and financial performance.

Data Controller (79.4%), Required Purpose (89.9%):

Benefits of athenaOneValue-Based Care Support Improve patient outcomes while tracking and getting paid for care.

Required Purpose (93.1%):

Advisory Services Expert consulting and support to help improve performance and ROI.

Retention (75.5%):

If our clients experience a penalty in 2016 for not attesting, we will credit them the penalty amount for the length of time that they experience the downward payment adjustment (not to exceed our services fees).

Non-Personal Data (67.5%), Processing (81.2%), Retention (80.0%):

12Time of Service Copay Collections Average total copay amount collected at the time of service divided by the total copay amount due by athenaCollector clients across athenaNet for February 2016.

Processing (83.3%), Retention (75.3%):

Referral-Related Denials: This data was gathered during a beta testing period for our athenaCoordinator Enterprise service, prior to full release to the general public.

Data Controller (63.7%):

Return to top III.References 6KLAS Report: "Interoperability 2015: Are We Lifting Together?" October 2015.

Third Party (81.6%):

Other health care trading partners and vendors may charge athenahealth clients for the creation, development, and maintenance of interface or health information exchange integration.

Data Source (80.0%), Personal Data (68.6%), Processing (94.2%):

Return to top IV.Tools PayerView: Payerview Disclaimer and Methodology PayerView rankings and data are based solely on actual claims activity captured in the athenaNet system from a limited set of physicians and payers.

Non-Personal Data (91.5%):

Rankings and data presented should not be relied upon or assumed to be representative of future experience, or of the experience of physician practices that do not participate in our services.

Data Source (78.6%), Non-Personal Data (83.5%):

Interoperability Get the data you need when you need it from across our network.

Required Purpose (72.6%), Retention (73.3%):

Payer & Performance: athenahealth defines "payer" as an organization to which the physician has to submit a claim in order to get paid.

Third Party (96.6%):

This includes a managed care organization, health insurer, military health plan, HMO, preferred provider organization, third party administrator, Medicare/Medicaid plan, and carrier or intermediary to whom physicians submit third party payment claims.

Processing (98.7%), Required Purpose (89.6%):

Some of these organizations may be forwarding claims to others for actual adjudication, processing, payment or other important steps in the payment process.

Processing (63.8%):

Since physicians normally cannot bargain for or dictate the terms of these relationships between payers, and since it may be difficult for physicians to track or monitor movement of claims reliably from one payer to another under these relationships, it is our opinion that the physicians vantage point is valuable, and we have therefore arranged the data to generally reflect "payers" to be those entities where claims are submitted by physicians in the first instance.

Processing (96.0%), Third Party (95.1%):

Similarly, the measures chosen for performance are affected by time lags, mistakes, processing practices and data quality on the part of the physician practices involved, by athenahealth itself and by third parties who act as intermediaries in the transmission of claims information.

Organisational Measure (88.1%):

We actively work to identify and to address matters that we or our customers can control that affect these and other measures, and we have adopted processes that differ from others in the industry.

Organisational Measure (62.2%), Required Purpose (82.3%):

Performance Metrics and Weighting: To set overall national and regional rankings, we weighted

each measure, placing a priority on fast, complete payment.

Retention (75.4%):

The weighting reflects our opinion as to the impact each measure has on the ability of athenahealth and its providers to receive payment on claims.

Retention (75.8%):

Days in Accounts Receivable (DAR): Average length of time it takes to receive payment as measured from the date of charge entry to the date of remittance post.

Processing (78.0%), Retention (60.7%):

This measurement encompasses the period from the day a charge is entered into athenaNet to the day payment for a charge is posted.

Retention (70.0%):

The more rework involved with a claim, the longer it takes for the provider to get paid and the greater the resources required by physicians to arrange payment.

Non-Personal Data (93.9%):

Ranking considers quantitative scores for enrollment in EDI, ERA, EFT, & PAYTO.

Non-Personal Data (89.7%):

Scores assigned based on enrollment type, signature requirements, and submission method.

Organisational Measure (62.1%):

This metric reflects how well the payer has adopted the standard code set by returning clear adjustment reason codes accompanied by remark codes as appropriate.

Required Purpose (80.8%):

Eligibility Accuracy: Correlation of eligibility response to adjudication outcome.

Processing (65.0%):

Note that PayerView does not measure the amount of money practices receive from payers; it quantifies, rather, the challenges practices face when collecting from payers for services.

Required Purpose (66.4%):

It is important for payers to return a reliable and detailed eligibility response so that providers can identify and resolve eligibility related issues before submitting the claim to the payer.

Non-Personal Data (80.8%):

For copays, this means having an exact match to the eventual copay amount.

Data Source (82.3%), Personal Data (91.6%), Processing (69.8%):

The information provided by PracticeVitals is based on historical data from a limited set of medical practices.

Required Purpose (91.2%):

Due to the inherent limitations of the data that modeling tools such as PracticeVitals must use, any information that they provide is offered for illustrative purposes only and cannot be expected or relied upon to project future results.

Data Subject (93.1%):

4Adoption Rates: Based on the percentage of athenaClinicals users who close more than 80% of encounters electronically (88% close all of their encounters electronically).

Required Purpose (98.9%):

The information provided by PracticeVitals does not constitute advice of any kind and should be viewed critically; users should obtain any additional information necessary to make an informed decision as to the selection of any product or service.

Organisational Measure (79.1%):

While athenahealth anticipates making regular changes to the data on which PracticeVitals is based, such changes may or may not be incorporated on a regular basis or at all; to the extent that changes are made, results provided by the tool may vary with each use and over time.

Personal Data (96.9%), Processing (98.4%), Required Purpose (79.2%), Retention (91.6%), Technical Measure (89.1%):

To avoid re-display of PracticeVitals introductory screens if you have recently used the tool, a cookie will be stored on your device for fourteen days to indicate your prior visit no personal information or settings are stored in this cookie; for more information on how information entered or stored when using our website and online tools may be used, please see our Privacy Policy, which is available at: <https://www.athenahealth.com/our-company/privacy-policy.php>.

Processing (95.4%):

athenahealth makes no representations, warranties, or guarantees regarding any information provided by PracticeVitals or the results obtained from its use or the use of any product or service and shall not be responsible or liable for any errors or omissions in that information or the results obtained from any such use.

Processing (70.7%), Third Party (78.6%):

10Encounter documentation time Average time a provider spends documenting in athenaClinicals during an encounter for athenahealth clients in 2014.

Required Purpose (94.8%):

Methodology: Our goal is to provide timely and specialty-specific market data for certain key operational metrics, together with best-practices suggestions, so that users can better evaluate and

improve the efficiency, productivity, and financial performance of their medical practices.

Retention (60.8%):

We therefore require a practice to have been using our services for that entire year plus another three months to control for potentially skewed data from practices just transitioning to our service to be considered for inclusion in the data set.

Non-Personal Data (62.2%):

Eliminating Spurious Data: To control for statistically anomalous data that could materially skew average practice performance, all figures shown are median values.

Retention (63.9%):

If a practice specialty is selected in the tool, data for practices with that specialty will be used unless the sample pool for that specialty is too small to be meaningful.

Processing (69.7%):

The rankings and data presented do not and are not intended to present or ascribe causes for that experience, and "performance" is intended to refer to measure of observed results when claims were submitted using our services to a payer.

Non-Personal Data (86.8%):

Unless otherwise specified, each metric represents the median statistic across all practices included in the data set for that metric.

Processing (92.3%):

As noted above in the definition of payer, the activity of a given payer may vary depending on the role that it assumes in the claim process; and, PayerView data results for a given payer may be caused in whole or in part by the actions or processes of others, including other payers, with whom or for whom the payer in turn conducts business.

Required Purpose (94.9%):

Metrics: The data described above are used to calculate the results in each of the following categories: No-Show Rate: No-Show Rate is calculated by dividing (1) a practice's total number of appointments cancelled with a Patient no-show reason during the month in question by (2) the total number of appointments scheduled for the same month for that practice.

Data Source (63.9%), Non-Personal Data (82.9%):

The information provided by the Revenue Calculator is based on historical data from a limited set of medical practices, average industry cost data published by others, and user responses to a few select questions.

Data Source (88.8%):

Revenue Calculator: Disclaimer: Use of the Revenue Calculator is subject to the following conditions and limitations.

Required Purpose (99.0%):

To improve the ease of use of the Revenue Calculator, any data or settings you submit will be retrievable on your computer for fourteen days through the use of a "cookie"; for more information on how information you enter when using our website may be used, please see our Privacy Policy, which is available at: <https://www.athenahealth.com/privacy-rights>.

Processing (80.1%):

If you have obtained information reportedly provided by PracticeVitals from a source other than athenahealth.com, please be aware that such electronic information can easily be altered subsequent to its original distribution.

Non-Personal Data (96.0%):

Source: iData Research (2008) Average Monthly Collections: The average monthly collections per provider, by specialty.

Non-Personal Data (69.0%), Personal Data (70.1%):

Source: athenahealth data (September 2010), athenahealth vendor survey (October 2010) Average Monthly Labor Cost: An estimate of the average monthly labor cost per provider for a medical records technician to process a practices internal documents for delivery to athenahealth.

Data Source (65.1%):

Source: athenahealth data (Year ended 2014) Storage & Management Cost Savings: The average annual practice savings in storage space and archive management costs due to EMR adoption, based on the number of practice physicians.

Processing (81.5%):

Source: iData Research (2008) Assumptions: In developing this calculator, we relied on our experience with our clients in assuming that a practice will generally have one label printer for every three providers and that a medical records technician will take between one and two minutes to prepare and send an internal document of the practice to athenahealth (documents from third parties are faxed directly to athenahealth without any action required on the part of the practice).

Third Party (71.8%):

Meaningful Use Attestation Rate: The "Providers who have attested" percentage compares the number of athenaClinicals providers who have successfully submitted their 2015 Medicare Meaningful Use performance information to athenahealth for registration and attestation with CMS to the total number of athenaClinicals providers actively pursuing Medicare Meaningful Use for a 2015

Medicare Meaningful Use program.

Processing (84.7%), Required Purpose (67.7%):

Who We Serve Provider Organizations Startup Practices Small Medical Practices Medium to Large Medical Groups Enterprises & Health Systems Federally Qualified Health Centers Member-centric Care Organizations Medical Specialties Behavioral Health Orthopedics Pediatrics Primary Care Urgent Care Women's Health View All Specialties Other Organizations Healthcare Payers Labs and Imaging Centers Partners Marketplace Partners Developers Resources Resources Blog Case Studies Customer Success Stories ebooks Our Thought Leaders View All Content WEBINAR The next era of behavioral health Catch the replay Company About athenahealth Who We Are Leadership athena Gives Join Us Contact Us Careers In the News Newsroom Press Releases See how athenaOne works Disclaimers Table of Contents Disclaimers Metrics References Tools Patents athenahealth ONC Certified Health IT I. Disclaimers Meaningful Use Guarantee: As a service-based EHR, our monthly fee is the only payment we receive from our clients for our EHR.

Required Purpose (62.6%):

Reduce No-Shows: To determine the number of additional visits each month due to use of athenaCommunicator, the entered number of monthly visits (as described in Increase Patient Volume) is multiplied by the Pre-athena No-show Rate and then the expected percentage improvement in the no-show rate.

Required Purpose (94.8%):

Patient Engagement Communication and care tools that improve patient relationships.

Required Purpose (74.5%):

To determine a provider's expected total Medicare payments for the year, the entered practice monthly collections is divided by the specified number of providers to obtain a per-provider average.

Required Purpose (66.0%):

ID NAME 1 Medicare Annual Wellness Visit 2 Influenza Vaccination 3 Colorectal Cancer Screening 4 Diabetes: Overdue HbA1C or LDL-C Screenings 5 Hypertension: Uncontrolled Blood Pressure 21 Breast Cancer Screening 22 Diabetes: Influenza Vaccination 23 Diabetes: HbA1c Control <7% 24 Adolescent Well-Care Visits 41 Blood Pressure Control for Adults (140/90) 42 Blood Pressure Screening 43 Cervical Cancer Screening 21-29 (Every two years) 44 Cervical Cancer Screening (Every 3 years) 45 Chlamydia Screening in Women 46 Pneumonia Vaccination 47 Shingles (Herpes Zoster) Vaccination 48 Pneumonia Vaccination for Asthmatics 49 CAD: Blood Pressure Control 50 CHF: Blood Pressure Control 51 IVD: Blood Pressure Control 52 Blood Pressure Screening for CAD 53 Lipid Control (Less than 100) for CAD 54 Lipid Process (1x year) for CAD 55 BP Control in

Diabetes (130/80) 56 BP Control in Diabetes (140/80) 57 BP Control in Diabetes (140/90) 58 Hb A1c Process (2x year) 59 Diabetes: HbA1c Control <8% 60 Lipid Control (Less than 100) 61 Pneumonia Vaccination for Diabetics 181 HPV Vaccination Initiation 182 HPV Vaccination Overdue 201 Well-Child Visit Aged birth-18 months 202 Well-Child Aged 18 months -3 years 62 Tdap Adult Booster for Diabetics 63 BP Control in Hypertension (140/90) 64 CKD4/CKD5 and diabetes: HbA1c < 7 65 CKD4/CKD5: Blood pressure control 66 Well-Child Visits Aged 3 - 11 67 Medicare Screening Pelvic Examination 68 Adult Annual Preventive Visit 69 Diabetes: Uncontrolled HbA1c > 9% or LDL-C >100 81 Tdap Adult Booster 101 Influenza Vaccination: Child 102 BMI: High-Risk Overweight or Obese 103 Diabetes: HbA1c Control <9% 104 Comprehensive Immunizations Before Turning 2 (HEDIS) 141 Blood Pressure Control: CVD or CHF or IVD 161 ACE/ARB monitoring Other QM Campaign CMS Incentive and Penalty Programs: Qualification for Meaningful Use penalties: For 2016, penalty total reflects the -2% reimbursement cut of the practices total Medicare Part B payments for the year as determined by CMS based on a providers 2015 performance.

Non-Personal Data (63.0%):

Source: athenahealth data (Q3 2015) Medicare Part B Percentage: The average percentage of collections that are received by practices under Medicare Part B, by specialty.

Processing (62.8%), Third Party (64.9%):

Calculations: The data and assumptions provided above are used to calculate the results in each of the following categories: Increase Collections: To determine the estimated increase in monthly collections due to use of athenaCollector, the Average Monthly Collections for a practice of the selected specialty (a per-provider average) is multiplied by the specified number of practice providers to estimate total practice monthly collections.

Third Party (76.9%):

www.klasresearch.com 8ICSA: The additional types of costs that could be incurred by a purchaser of this system include: athenahealth's online implementation service is free for practices with one to six physicians.

Required Purpose (81.6%):

This number is then divided by twelve to obtain a monthly average document services cost savings.

Non-Personal Data (69.3%):

of Labor (Year ended May 2009) Average Monthly Printer Cost: The average price for a label printer, amortized over a year.

Non-Personal Data (84.8%):

Source: athenahealth data (Q3 2015) Average Monthly Label Cost: The average price per provider

for printed labels used to identify clinical documents faxed to athenahealth during one month.

Non-Personal Data (68.3%):

Return to top II.Metrics 1Increase in Collections, Decrease in DAR: Our clients see an average 6% increase in collections and 32% decrease in days in accounts receivable, or DAR, based on a weighted average for athenahealth clients with valid pre-athenahealth benchmark data that had their 15 month anniversary with athenahealth between January 1, 2010, and October 31, 2013.

Non-Personal Data (90.7%):

The information provided by the Full Value Calculator is based on historical data from a limited set of medical practices, industry-set penalty and incentive qualifications, and user responses to a few select questions.

Required Purpose (66.8%):

Due to the inherent limitations of the data that modeling tools such as the Full Value Calculator must use, any information provided by the Full Value Calculator is offered for illustrative purposes only and only offers approximate estimates based upon certain historic data, as described above.

PAD (100.0%):

First Pass Resolved claims are classified as receiving full or partial payment and/or deemed as patient responsibility by the payer on the initial claim submission.

PAD (100.0%):

www.KLASresearch.com *Interoperability 2017: First Look at Trending Some progress toward a distant horizon October, 2017.

PAD (99.4%):

Athenahealth undertakes no obligation to provide error-free or fault-free items or services, and the VBR Calculator are provided as is with all faults and defects.